

UC85 Work Capability Assessment Report

Claimant: Michael thompson

Date of Assessment: 09 June 2026

Assessment Type: Face-to-Face Assessment

Health Conditions Considered:

- Chronic back pain (10 years)
 - Osteoarthritis affecting shoulders, knees and hands (4 years)
 - Depression (3 years)
 - Ischaemic heart disease following myocardial infarction (7 years ago)
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Medical History

Mr Hughes reports longstanding mechanical back pain with stiffness and reduced mobility. Symptoms are worsened by prolonged standing, walking and bending. He experiences daily pain despite regular analgesia.

He reports osteoarthritis affecting both hands, shoulders and knees. Hand pain and reduced grip affect fine manual tasks including fastening buttons and handling small objects. Knee pain causes reduced walking tolerance and he uses a walking stick outdoors.

Mr Hughes suffered a myocardial infarction seven years ago. He remains under GP management and reports exertional angina relieved by rest and GTN spray. He reports that excessive walking can precipitate chest pain.

Mr Hughes also reports depression treated with Sertraline. He reports reduced motivation and low mood but remains able to engage appropriately during the assessment.

Current medication includes GTN spray, Co-codamol, Naproxen, Isosorbide Mononitrate, Lansoprazole, Sertraline, Atenolol, Atorvastatin and Aspirin. No significant medication side effects reported.

GP last reviewed the claimant one month prior to assessment. [\[work-capab...fessionals | Word\]](#)

Social and Occupational History

Mr Hughes lives with his partner in a two-storey property.

Previously employed in manual work but reports being unable to continue because of worsening musculoskeletal pain and reduced exercise tolerance following cardiac disease.

Travelled to the assessment centre by car driven by his partner. Uses a walking stick when outdoors.

Typical Day

Mr Hughes reports rising independently but requiring additional time due to pain and stiffness.

His partner undertakes the majority of cooking. He reports difficulty chopping vegetables and preparing meals because of reduced grip caused by arthritis in his hands. He assists with simple tasks where possible.

Medication is organised by his partner due to difficulties handling tablets and blister packs.

He reports bathing independently but requires additional care entering and leaving the bath. He uses a non-slip mat and steadying support due to concerns about balance and mobility.

Dressing takes significantly longer than previously. He reports difficulty managing shirt buttons and requires assistance from his partner.

He remains able to eat and drink independently.

For mobility he reports using a stick outdoors. Walking distances are variable but limited by knee pain, back pain and angina symptoms. He reports needing regular stops and pacing himself.

Finances are largely managed by his partner.

Observed Behaviour

Mr Hughes attended appropriately dressed.

He mobilised slowly from the waiting area using a single walking stick. Gait was mildly antalgic. No acute distress observed.

He rose from sitting independently but with reduced speed. During the assessment he altered position occasionally and reported discomfort in his lower back.

He demonstrated adequate communication and comprehension throughout.

Clinical Examination

Lumbar Spine

Reduced forward flexion to approximately knee level due to pain.

Upper Limbs

Reduced grip strength bilaterally consistent with reported osteoarthritis affecting the hands.

Shoulder movements mildly restricted bilaterally.

Lower Limbs

Knee movements reduced by pain and stiffness. No significant muscle wasting noted.

Cardiovascular

No evidence of acute cardiovascular compromise at assessment. History remains consistent with exertional angina limiting exercise tolerance.

Functional Assessment

Activity 1: Mobilising

Evidence supports restricted mobilising ability due to combined effects of osteoarthritis, chronic back pain and ischaemic heart disease. Mr Hughes reports requiring a walking stick and experiencing angina on exertion. Typical day evidence indicates he self-limits activity and requires rest breaks.

Descriptor chosen: Cannot either mobilise more than 200 metres on level ground without stopping in order to avoid significant discomfort or exhaustion, or repeatedly mobilise 200 metres within a reasonable timescale because of significant discomfort or exhaustion.

Score: 6 points

Activity 2: Standing and Sitting

Able to remain seated for periods exceeding one hour with position changes.

Descriptor: None apply.

Activity 3: Reaching

Mild bilateral shoulder restriction present but retains ability to reach above shoulder level.

Descriptor: None apply.

Activity 4: Picking Up and Moving

Able to lift and transfer light household objects.

Descriptor: None apply.

Activity 5: Manual Dexterity

Arthritic hand symptoms cause difficulty with buttons and fine dexterity tasks but claimant remains able to use a pen, telephone and manage larger objects.

Descriptor: None apply.

Mental Function Activities

Depression noted but claimant demonstrated appropriate concentration, memory, engagement and communication throughout assessment.

Descriptors: None apply.

Personal Summary Statement

Mr Hughes has chronic musculoskeletal conditions affecting the spine, knees, shoulders and hands together with stable cardiac disease following myocardial infarction. Clinical history, examination findings and reported daily activities support significant limitations in mobility due to pain, stiffness and reduced exercise tolerance.

He remains independent in many personal care activities but requires assistance from his partner with medication management, meal preparation and fastening clothing due to reduced hand function.

Observed mobility with a walking stick, combined with his account of exertional angina and osteoarthritic knee pain, support a significant restriction in mobilising ability. However, evidence does not suggest severity consistent with Limited Capability for Work-Related Activity criteria.

Mental health symptoms are present but do not currently produce significant restrictions across the mental function activities.

Overall, the available evidence supports Limited Capability for Work.

Advice to Decision Maker

Limited Capability for Work (LCW): Yes

Limited Capability for Work and Work Related Activity (LCWRA): No

Recommended Review Period: 24 months

Justification: Chronic degenerative musculoskeletal conditions and cardiac disease are likely to remain present. Functional improvement is unlikely in the short term though further deterioration cannot be excluded. Reassessment in approximately two years is appropriate.